

SOUTH AFRICA'S PRIMARY HEALTH CARE SYSTEM

Group 3F



UCL



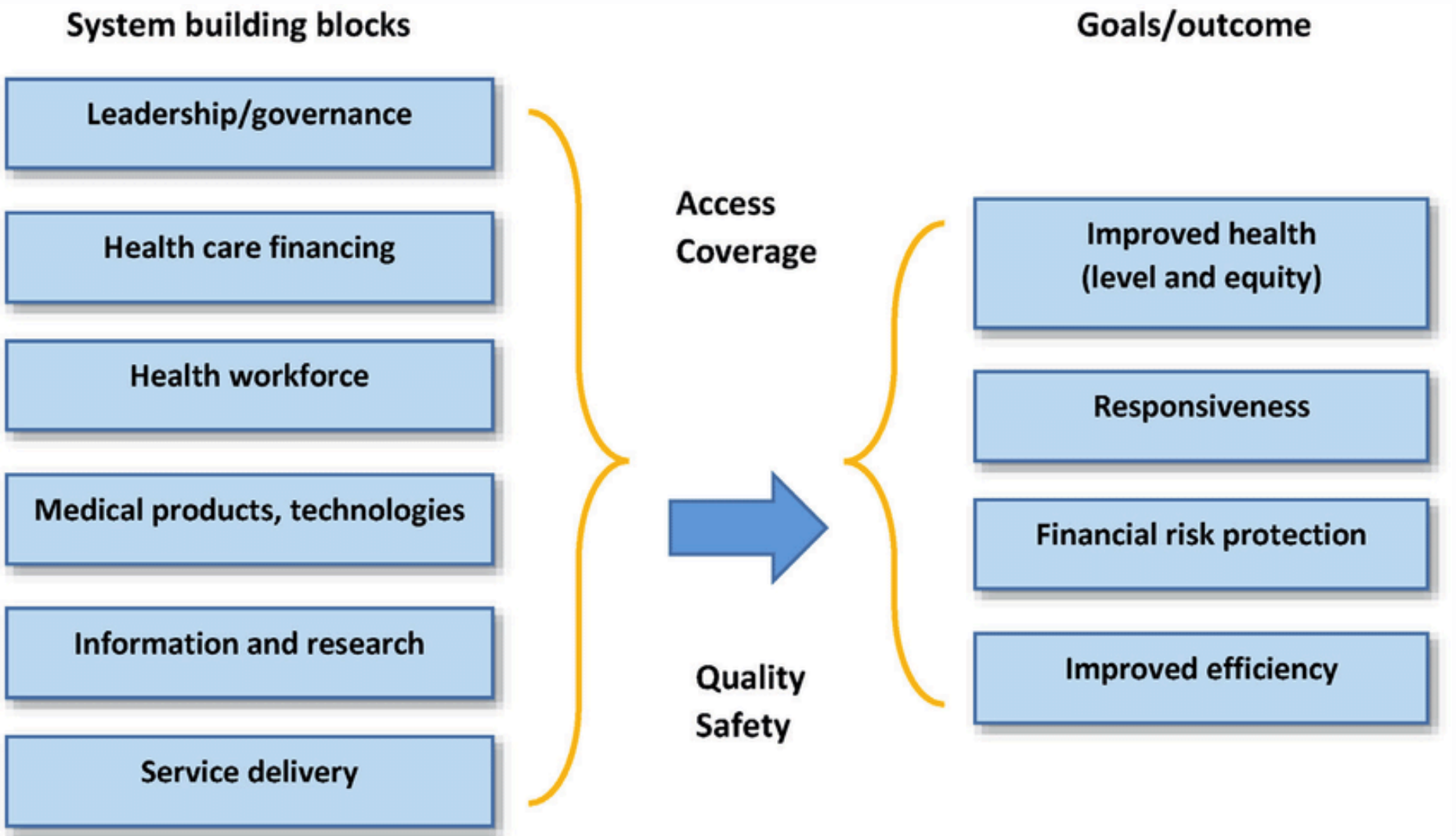
Key Milestones in Healthcare Reform

1978: Alma Ata Declaration – Global commitment to Primary Healthcare (PHC) for all.

1997: Health White Paper – Established equitable, decentralized healthcare policies.

2015: NHI White Paper – Laid the groundwork for Universal Health Coverage (UHC).

2024: NHI Bill Signed – Marked a new era of healthcare accessibility and equity.



WHO Organization, 2007.

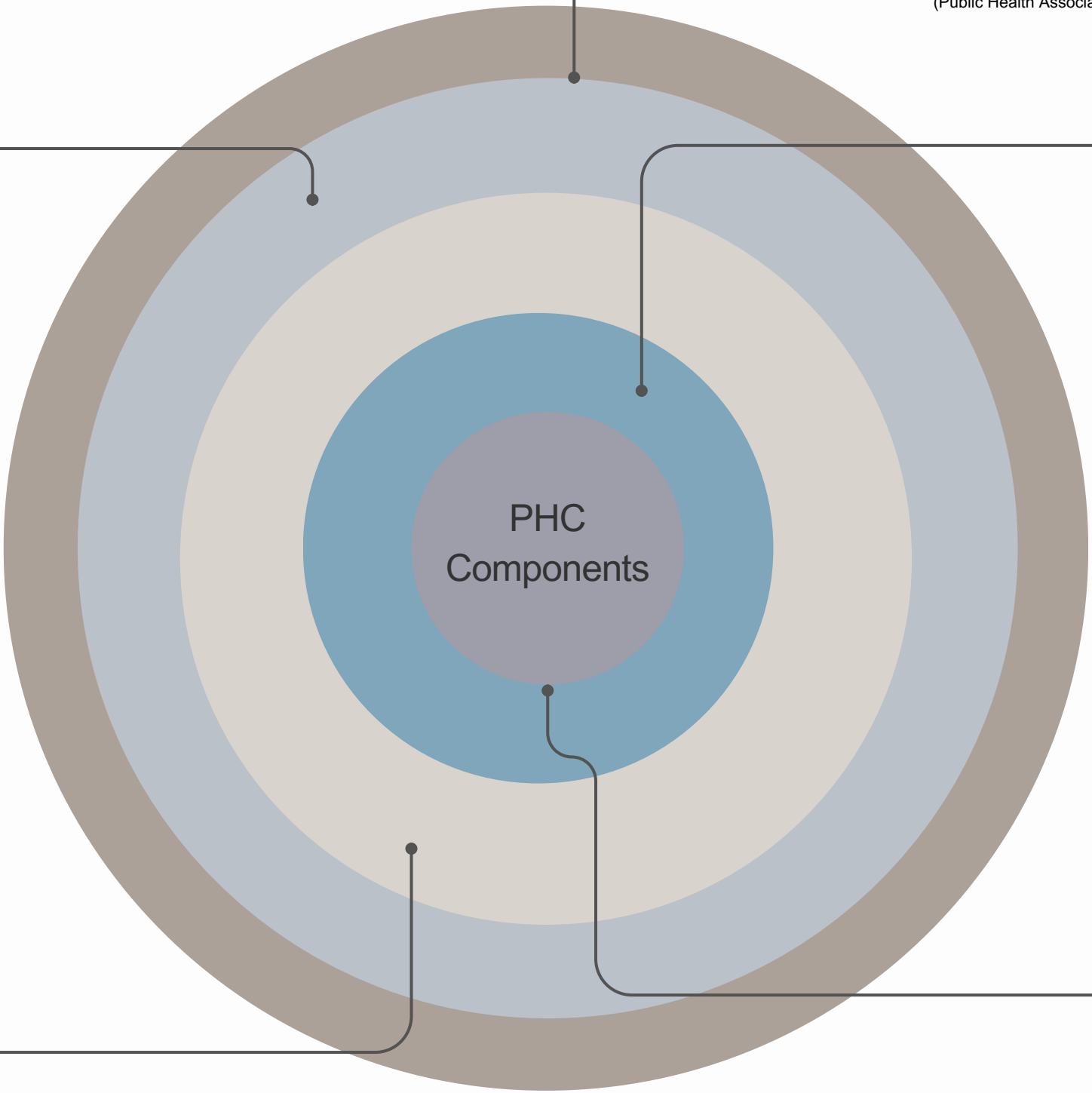
(Naledi, Barron, and Schneider, 2011)

PRIMARY HEALTHCARE SYSTEM



GOALS: INCREASING LIFE EXPECTANCY, DECREASING MATERNAL AND CHILD MORTALITY, COMBATING HIV AND AIDS, DECREASING THE BURDEN OF DISEASE FROM TUBERCULOSIS, AND STRENGTHENING HEALTH SYSTEM EFFECTIVENESS IN ITS PHC SECTOR.

(Public Health Association of South Africa, n.d.).



NURSE-DRIVEN LEADING 80% OF HEALTHCARE PROVIDERS IN RURAL AREAS ARE NURSE PRACTITIONERS.

84% RELY ON THE PUBLIC SECTOR.

16% RELY ON THE PRIVATE SECTOR.

(World Health Organization, n.d.).

COMMUNITY HEALTH WORKERS: INTRODUCED TO IMPROVE PRIMARY HEALTHCARE BY MANAGING DESIGNATED POPULATIONS.

TOTAL HEALTH EXPENDITURE
9% OF GDP IS SPENT ON HEALTH, WHICH EQUALS US\$ 570 PER PERSON.

PUBLIC SPENDING:
48% OF TOTAL HEALTH SPENDING.

PRIVATE SPENDING:
43% OF TOTAL HEALTH EXPENDITURES.

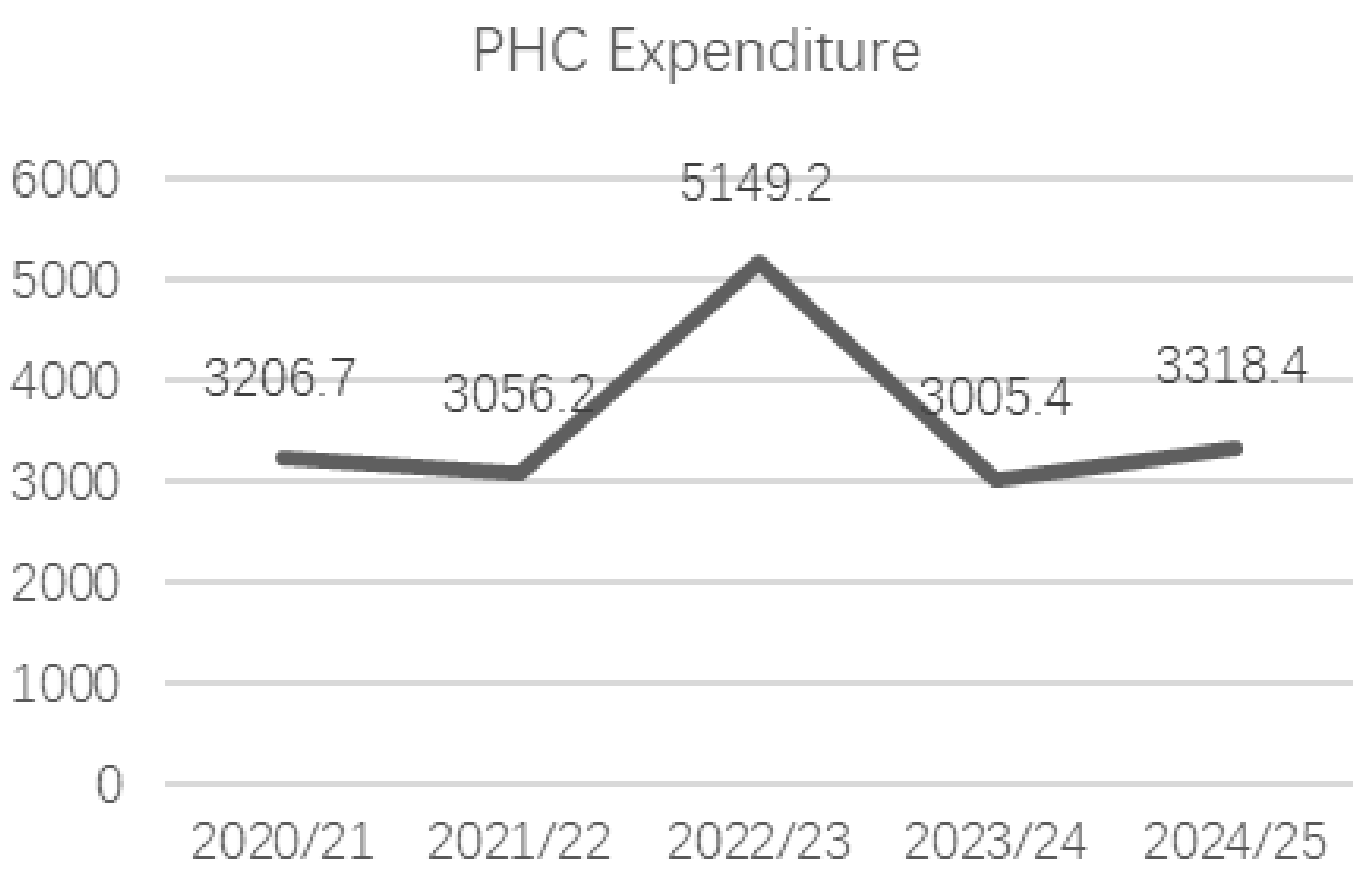
(Mahlathi & Dlamini, 2019).



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PHC	Audited Outcome			Expenditure Estimate	
Sub-program	2020/21	2021/22	2022/23	2023/24	2024/25
Program Management	3.5	4	4.5	6.9	6.8
District Health Services	2905.7	2819.1	4906.4	2951.1	3258.3
Environmental and Port Health Services	290.6	226.4	229.3	38.8	44.1
Emergency Medical Services and Trauma	6.8	6.7	9.1	8.6	9.1



(National Treasury Republic of South Africa, 2019-2024. 2023/24 and 2024/25 are budget.)

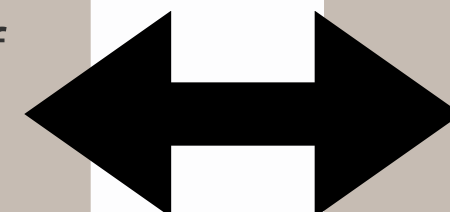
Key Issues:

- **Unstable expenditure:** Affects the sustainability of long-term medical resources.
- **Unstable funding allocation:** Leads to inadequate resource allocation and supervision / affecting the ability to respond to emergencies
- **Smaller budget increases:** Potential cuts in the quality of medical services
- **Insufficient funds:** Difficult to provide quality service (Abrahams et al., 2022)
- **Financial management levels vary:** Affects the effective use of funds (Hanson et al., 2022)



Contextual Factors

- **Macroeconomic Instability:** Slow economic growth, high unemployment, and significant fiscal constraints (CIPRO, 2024)
- **Insufficient financing for UHC:** Fiscal sustainability of NHI (Solanki et al., 2024)
- **Fragmented Health System:** The dual health system (public vs. private) leads to unequal access (de Villiers, 2021)
- **Corruption and Inefficiencies:** mismanagement, fraud, and corruption further deplete health funds (Jarbandhan et al., 2023)



Recommendations

- **Decentralize budget control with capacity building:** Provide financial management training for district-level health officials (Marten et al., 2021).
- **Expand public funding support:** Improve infrastructure construction and human resources support
- **Protect PHC funds:** Prevent PHC funds from being reallocated to other sectors.
- **Accelerate National Health Insurance (NHI) rollout:** Implement universal coverage policies (National Department of Health, 2021)





KEY ISSUE 1

Financial Mismanagement & Corruption

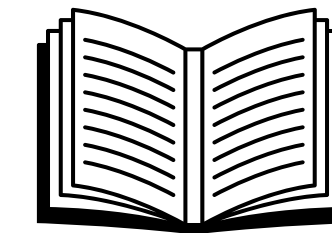
- Fraudulent procurement (Mokoena and Naidoo, 2024) → Overpriced contracts and misallocated funds.
- Decentralized budgeting (Murphy and Moosa, 2021) → Weak provincial oversight.



KEY ISSUE 2

Disparities Between Public & Private Healthcare

- Private sector dominance (McIntyre, 2019) → A few hospital groups control pricing.
- Limited affordability (Ataguba and Goudge, 2021) → 84% of South Africans rely on underfunded PHC.



CONTEXTUAL FACTORS

- Extreme Income Inequality (Ataguba and Goudge, 2021) → Public system overburdened, private care expensive.
- Regulatory Gaps (Mokoena and Naidoo, 2024) → Weak enforcement of financial controls.



1. STRENGTHENING FINANCIAL OVERSIGHT & TRANSPARENCY

- **Digital Financial Tracking & Open Budgeting** (National Department of Health, 2023)
→ Prevents fund misallocation.
- **Independent Anti-Corruption Audits** (Mokoena and Naidoo, 2024)
→ Ensures provincial accountability.



GOAL
EFFICIENT

ACCOUNTABLE

EQUITABLE
HEALTHCARE
FINANCING

2. REGULATING PRIVATE HEALTHCARE & ADDRESSING MARKET FAILURES

- **Price Caps on Essential Services** (WHO, 2020)
→ Prevents cost inflation.
- **Expand Public-Private Partnerships (PPPs)** (Murphy and Moosa, 2021)
→ Contracts private GPs for PHC.





INPUTS

Inputs: Available
& Accessible

- Health financing
- Human resources
- Physical facilities
- Policies and guidelines



PROCESSES

Processes: What is done

- Management of health services



OUTCOMES

Outputs

- Unequal Healthcare Resource Distribution
- Shortages of health care workers

Impacts

- Increased morbidity
- Increased mortality



Key Issue

1. Unequal Healthcare Resource Distribution

Access to health care is a particular concern given the important role of poor access in perpetuating poverty and inequality (McLaren et al., 2014). In South Africa, PHC is unevenly resourced, and the delivery of services is fragmented (WHO, 2017).

- **Ethical inequality:** The legacy of apartheid leaves non-whites in remote areas, which are potentially underserved (Harris et al., 2011)
- **Service Fragmentation:** Disparities between public and private healthcare; a well-resourced, insurance-based private sector serving only **16%** of the population but consuming **half the total funds flowing**

2. Shortages of health care workers

low HCW-population ratio

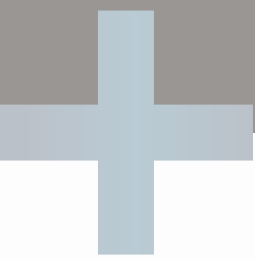


Chain Reaction

- **High Workload:** Longer working hours and low job satisfaction (Phaswana-Mafuya et al., 2008).
- **Burnout:** Physical and mental exhaustion (Howard et al., 2019).

≈ 80%

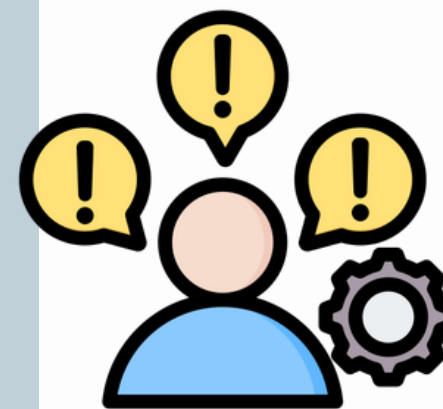
Private sector



Contextual Factors

Within Health System

- **Public vs. Private Sector Divide:** A **dual health system** exists
- **Historical Inequities:** The legacy of apartheid → Struggling with inadequate resources
- **Limited Education & Training:**
Underinvestment in education → shortage of new medical staff (Mbemba et al., 2016)
- **Low Salaries & Poor Working Conditions** → stressful and inefficient (McLaren et al., 2014)
- **Limited Career Advancement** (Howard et al., 2019)



Outside Health System

- **Geographical Disparities:** Transportation challenges in rural areas → considerable costs of access (WHO, 2017)
- **Global Demand for Healthcare Workers:**
Increasing demand → more option
- **Economic Constraints:**
 - **1/3** of the population lives in rural areas, where an estimate of poverty exceeds **70%** (WHO, 2017)
 - Difficult to provide adequate funding → Affect recruitment and retention (McLaren et al., 2014)
- **Social Perception & Recognition:** Young people prefer High-paying industries (e.g. IT, Finance) (Mayeng & Wolvaardt, 2015)



Recommendations

1. Improve accessibility & reduce workload

1. Public-Private Collaboration

- Foster **partnerships** ➔ expand access to specialized care
- Shared-service **agreements**



2. Equitable Redistribution of Healthcare Workers

- Introduce **rural retention** (e.g. housing, education grants and financial bonuses)
- Strengthen rural **training programs**

3. Task-Shifting

- **Expand and formalize the role** of community health workers (De Kock & Pillay, 2016).
- Develop clear **competency-based certification programs** for mid-level providers
- Integrate task-shifting policies into **national workforce planning**

Accessibility

Workload

2. Governance & Quality Assurance

- **Implement national quality standards** for both public and private institutions (Howard et al., 2019)
- **Develop a centralized data system**
 - Monitor workforce distribution
 - Track patient outcomes
 - Evaluate service efficiency
- **Strengthen patient engagement and feedback systems**
 - Rebuild trust
 - Reflect community needs

Quality



Slide 2	PBSY1
Slide 3	PBSY1
Slide 4	NNVS4
Slide 5	NNVS4
Slide 6	QCFM4
Slide 7	QCFM4
Slide 8	PLZS0
Slide 9	PLZS0
Slide 10	QPPB2
Slide 11	QPPB2



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